Department of Media Studies and Journalism

University of Liberal Arts Bangladesh
Final Project| Fall Semester, 2025
MSJ 3263: Emergency Communication (Sec: 01)

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Students' ID	231012029, 223012075, 231012010
Section	
Total Marks	**100**
Marks obtained	

Problem Statement and Description:
Emergency and risk communication refers to the process of conveying critical information to the public during times of crisis or potential danger. It involves disseminating accurate, timely, and actionable information to help individuals, communities, and organizations make informed decisions and take appropriate actions to protect themselves and mitigate risks. The goal of emergency and risk communication is to ensure that people have the knowledge and resources they need to respond effectively to emergencies and minimize potential harm. This includes providing information about the nature of the emergency, the potential risks involved, recommended safety measures, evacuation procedures, and available support services. Effective emergency and risk communication requires a multi-faceted approach that involves various communication channels, such as mass media, social media, community outreach, and direct messaging. It also emphasizes the importance of clear, concise, and consistent messaging to avoid confusion and misinformation. Keeping in mind, prepare a report on emergency communication.

Bangladesh frequently faces emergencies such as floods, cyclones, heatwaves, disease outbreaks, industrial accidents, and humanitarian crises. Effective emergency communication is critical for saving lives, reducing panic, building trust, and enabling coordinated response.

This assignment assesses students' ability to design a clear, ethical, and evidence-based emergency communication plan, grounded in international best practices and adapted to the Bangladeshi socio-cultural context.

Learning Objectives
By completing this assignment, students will be able to:

- Explain key principles of emergency and risk communication
- Design communication strategies for different phases of an emergency
- Develop clear, actionable, and trustworthy emergency messages
- Identify stakeholders and coordination mechanisms
- Integrate monitoring, feedback, and rumor management
- Apply ethical standards in crisis communication

Target Audience:
Senior managers of a development organization, donor agencies, government officials

Goals and Expectations:

- You will be able to prepare a report on emergency and risk communication
- Your plan should be original

Constraint:
There is no constraint in preparing the plan.

Deliverables:

You have been hired as the lead Emergency Communications consultant for a major city corporation such as Dhaka/Rajshahi/ Chattogram /Sylhet/Rangpur/Barishal. Your task is to develop a comprehensive Emergency Communications Plan that addresses key issues and best practices.

Assignment Title
Designing an Emergency Communication Plan for Crisis Preparedness and Response in Bangladesh

Assignment Task
Students will design a comprehensive Emergency Communication Plan for one type of emergency in Bangladesh, covering preparedness, response, and early recovery phases.

Emergency Scenario (Choose One)

- Cyclone or flood emergency
- Dengue, cholera, or pandemic outbreak
- Heatwave or climate-induced disaster
- Fire or industrial accident
- Rohingya refugee emergency
- Earthquake (low probability, high impact)

Required Structure of the Assignment
Section 1: Emergency Context and Risk Analysis

- Description of the emergency
- Risk and vulnerability analysis
- Affected population groups
- Communication challenges (panic, misinformation, access)

Section 2: Principles of Emergency Communication (15 marks)
Explain and apply key principles, such as:

- Risk Communication and Community Engagement (RCCE)
- Timeliness, transparency, and trust
- Two-way communication
- Do-no-harm and accountability
- Coordination and consistency

(Reference WHO, UNICEF, IFRC, Sphere standards)

Section 3: Communication Objectives and Audiences

- Communication objectives for:
    - Preparedness
    - Response
    - Early recovery
- Audience segmentation:
    - General public
    - High-risk groups
    - Responders and institutions

Section 4: Message Development and Framing
Students must develop:

- Key life-saving messages
- Message framing (simple, actionable, culturally appropriate)
- Language and literacy considerations
- Rumor and misinformation management strategies

Include sample messages.
Section 5: Channels and Coordination Mechanisms

- Communication channels:
    - SMS, radio, TV
    - Community volunteers
    - Social media and digital platforms

- Coordination with:
    - Government agencies
    - NGOs and media
    - Community leaders

Section 6: Monitoring, Feedback, and Adaptation (10 marks)

- Indicators for message reach and understanding
- Community feedback mechanisms
- Media and rumor monitoring
- Adaptive communication actions

Section 7: Ethics, Inclusion, and Accountability
Discuss:

- Protection of vulnerable groups
- Avoiding fear, stigma, and blame
- Accessibility (language, disability, gender)
- Accountability to affected populations (AAP)

Section 8: Supplement

- Message flowchart for different emergency phases
- Sample SMS or radio script
- Crisis communication timeline (first 72 hours)

Submission Guidelines

- Length: 2,500–3,000 words
- Format: MS Word or PDF
- Font: Times New Roman, 12 pt, 1.5 spacing
- Referencing: APA style
- Minimum references: 8 (WHO, UNICEF, IFRC, academic sources)

Assessment Rubric (Summary)

Criteria	Marks
Risk analysis and context	15
Application of emergency communication principles	20
Quality of messages and strategy	20
Channel selection and coordination	15
Monitoring and feedback	10
Ethics and inclusion	10
Structure and clarity	10
Total	100

Academic Integrity
All submissions must be original. Plagiarism or unethical use of AI tools will be handled according to university policy.

Submission deadline:
The deadline for submitting the plan is December 25, 2025

Keep in Mind:

- You must **read the instructions carefully** before writing your answer.
- You should download the question paper, and write your answer at the recommended place.
- Total Marks: 100 [will weigh 30% in the final grading]
- You must write your assignment in English, and your answer must be computer-composed.
- Deadline of Submission: December 25, 2025
- This is a group assignment project. So, you should form a group 2 to 3 individuals. All members of the group should submit their soft copy of the assignment. Submit only a hardcopy from the each group.
- You must submit a soft copy of your assignment to the Google Classroom using your account. **No Email submission will be accepted**

- **You will also submit a hard copy of your assignment at the first class after the midterm exam week, and get signed your exam attendance.**
- **While submitting hardcopy, please print this question paper also.**

- You may consult published academic journals, articles, books etc. In that case, you must write it in your language and give proper citations*. Any plagiarism will be considered a punishable offence, and it will severely affect your grades.
- You must submit your articles in a **single MS Word file**. You MUST NOT send your assignment in PDF or other file formats.
- The **name of your submission file** should be as follows: [Your Name] _ [Your Student ID] _ [ER– Section 1] _ [Fall 2025].
- You will use Times New Roman Font.
- Keep a one-inch margin on each side and line space 1.5 inches. You will submit all the answers in **ONE** file. Keep in mind, NOT more than one file.
- The Course Teacher will run a Similarity-Check in Turnitin. Papers with a Similarity index higher than 20% will not be accepted.
- If the answer of two or more students matches completely, or certain paragraphs used in the answer exactly match, all the students having such similarity will get ZERO. No explanation will be accepted in this regard.
- For any guidance, do communicate with me. You can contact me at 01718674830 / aminul.islam@ulab.edu.bd

***Citation should be as follows:**
In the text: (Author Name, publication year)
In Reference: Author name, (publication year). Title of the article, *Name of Journal*/book, Issue, Volume, page numbers, [for book: publication place: Name of publisher**].**

[You should begin writing from you're here]

Dengue Outbreak Emergency Communication Plan

Description of the emergency:
Bangladesh is currently facing a deadly dengue outbreak around the country. The sheer volume of cases in 2025 has surpassed the historic peak. The situation is critical due to the unpredictable shift in where and when infection is happening. This year the outbreak stretched into the winter months(late December) defying traditional seasonality(*WHO*, 2023)

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Key risk factors:

- Geographical expansion: Unlike previous years the sole epicenter was Dhaka city, 2025 the outbreak aggressively spread to Barishal, Chattogram, and Khulna divisions. Rural areas are now new hotspots where healthcare infrastructure is fragile.

- **Serotype Shift (Re-infection Risk):** The circulation of **DENV-2 and DENV-3** serotypes is driving severe cases. Populations previously infected with one strain are now contracting a different one, leading to *Antibody-Dependent Enhancement* (ADE)—a condition that significantly increases the risk of Dengue Hemorrhagic Fever and Shock Syndrome, Billah, Mallick & Shampa, Nazmun & Henderson, Alden. (2025).

- **Climate factors:** Due to climate change monsoon season has extended with irregular rainfall and warmer than average Winter has kept vector breeding active into December.

- **Healthcare strain:** Surging dengue patients have overwhelmed the tertiary care hospitals, as they have limited infrastructure to treat that many patients (Mahedi et al., 2025)

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Vulnerability Analysis: who is at most risk
The virus is affecting demographics differently. While more cases are recorded in one group, deaths clustering are in another.

Population Group	**Risk Profile**	**Why? (Vulnerability Drivers)**
Young Adults (Age 16–30)	**High Infection Risk**	This is the most mobile demographic (work/study). They are highly active during peak mosquito biting hours (early morning/late afternoon) in schools, universities, and offices.
Women (All Ages)	**High Mortality Risk**	**Critical disparity:** While fewer women are hospitalized than men, they have a **higher Case Fatality Rate (CFR)**. This is often due to delayed care-seeking behaviour (household prioritization) and social barriers to accessing immediate emergency transport.
Children (0–15)	**High Severity Risk**	Children's immune systems are less robust against shock. Warning signs (like rapid pulse weakening) can escalate much faster in children than adults.
Rural Residents	**Systemic Risk**	Residents in newly affected districts (e.g., Barguna, Pirojpur) often lack access to platelet separators or ICUs nearby, necessitating dangerous long-distance travel during critical illness phases(*WHO*, 2023) .

Communication Challenges:

Effective response is currently disrupted by three major communication barriers.

- **The platelet panic Fluid**

- The miscommunication: People panic when platelet drops below 100k and demand unnecessary transfusions (Billah et al., 2025)
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- Reality: Death is rarely caused by low platelets alone; it is caused by plasma leakage from the blood vessels, leading to shock.
- Challenge: Shifting the public narrative from “watch the platelets” to “watch the blood pressure and urine output.

- **The seasonal myth**

- The miscommunication: It's December (winter month) so the fever can not be dengue
- The risk: This belief causes the patients to disregard the early symptoms as the common flu. They delay testing and enter the critical phase.

- **Reliance on home remedies:**

- The miscommunication: Some people have the idea that papaya leaf is a cure to dengue.
- The risk: Relying on these remedies often delays necessary medical monitoring.

Explaining and implementing the principles of Emergency Communication:

- **Timelines, Transparency, and Trust:**

  Information must be released immediately, acknowledging the uncertainty to build credibility.

- The Challenge: The public belief is that dengue is a monsoon disease. Fever in the winter season is dismissed as a cold, delaying the treatment.

Application:

- **Transparency:** Authorities must (DGHS) clearly state: This is not normal. The season has shifted. Mosquitoes are surviving in the winter.
- **Timelines:** Do not wait for confirmation of the PCR test to warn a community. If cases of fevers appear immediately, issue a provisional alert.
- **Build trust:** Use trusted messengers like Imams and school teachers. People trust local figures who share their daily lives.

- **Two way communication(listening more than telling):** RCCE is not a monologue. We must set up channels to listen to community fears and myths to address them effectively.

- **The Challenge:** “The Platelet Panic” Patients’ families harass doctors for platelet transfusions (which are rarely needed) while ignoring the real killer: **blood pressure drop (shock)**.

Application:

- **Feedback Mechanism:** Monitor social media posts, community groups, and set up dedicated ‘Dengue Myth Hotline’ hospitals; if the dominant conversation is about platelets, do not ignore it.

Response: We hear you worrying about the bleeding. But it is not the only reason for the deaths due to dengue. In this new strain, plasma leakage is the silent killer. The IV saline is the right medicine that saves you from the shock. Watch the urine output, not just the blood count.
Visual Aid: Use simple infographics in the local dialect to explain to people

Do-not-harm and Accountability: Interventions should not inadvertently damage the community, stigmatize groups or drain their resources.

- **The challenge:**
- **Stigma**: Promoting slum areas as dirty and dengue zones stops residents from seeking help or allowing volunteers inside.
- **Financial Harm**: Private clinic leveraging the panic to sell expensive dengue test, platelet kits to poor families.

Application:

- **Accountability**: Publicize the official government price for the dengue test by attaching posters in the public hospitals and also on social media. Also, provide a number for complaints.

Coordination and Consistency: Conflicting messages can cost lives. Every actor should speak with one voice.

- **The Challenge:** A doctor in Khulna prescribes antibiotics, while a doctor in Dhaka prescribes fluid. The patients got confused and delayed treatment.
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Application:

- **Unified Protocol:** The DGHS, Bangladesh Pediatric Association, and NGOs must use the exact same one-page fever management guide.

Reference used:
1. WHO Outbreak Communication Guidelines (2005)
2. IFRC Community Engagement and Accountability (CEA) Policy,
3. Sphere Health Standard 2.2.1 (Control of Communicable Diseases)

Communication Objectives and Audience Segmentation:

1. Communication Objectives: The "What & When"
Phase 1: Preparedness (Before/Early)

- **The Goal:** Break the "Winter Myth."
- **What they must KNOW:** "Mosquitoes are hiding *inside* your warm house (fridge trays, flower vases) this winter."

- **What they must DO:** "Inspect your home every Friday morning. Pour out any water found indoors."

Phase 2: Response (NOW)

- **The Goal:** Stop "Platelet Panic" and start "Fluid Focus."
- **What they must KNOW:** "Death comes from **low blood pressure (shock)**, not low platelets. Saline is the medicine."
- **What they must DO:** "Go to the hospital immediately if the patient has **Warning Signs** (severe tummy pain, vomiting 3+ times, no urine for 6 hours)—even if fever is gone."

Phase 3: Early Recovery (After)

- **The Goal:** Restore trust in local care.
- **What they must KNOW:** "Local hospitals have the right saline protocols. Travelling to Dhaka while in shock is dangerous."
- **What they must DO:** "Support community clean-up of water tanks to prevent next year's surge."

2. Audience Segmentation: The "Who & How"

Audience Group	Who are they?	Precise Message (The "Script")	The Action Required
The "Gatekeepers"	**Mothers & Housewives** (They care for the sick)	"Don't wait for bleeding. Watch the urine. If the patient stops peeing, run to the hospital."	**Monitor:** Urine output & hydration.
The "Vulnerable"	**Rural Residents** (Barishal, Khulna, Pirojpur)	"Do not rush to Dhaka immediately. The long journey can kill. Stabilize at your Upazila Health Complex first."	**Trust:** Local stabilization before transport.
The "Ignored"	**Pregnant Women**	"A fever for you is a double emergency. Do not stay home. Admit to hospital on Day 1."	**Admit:** Immediate hospitalisation.
The "Enforcers"	**Pharmacy Owners** (The first point of contact)	"If a customer has fever, **DO NOT sell painkillers** (NSAIDs like Diclofenac). Sell Paracetamol only."	**Refuse:** Stop selling harmful drugs.
The "Influencers"	**Imams & Teachers**	"Cleanliness is faith. This Friday, tell everyone to clean their fridge trays."	**Preach:** Specific cleaning habits.

Action charts for responders and institutions
Category A: Community & First Responders
These actors control the spread and initial triage.

Role / Institution	**The Critical "STOP" (Do Not Do This)**	**The Critical "DO" (Mandatory Action)**	**The Script (What to say)**
Pharmacy Owners & Staff	**STOP selling NSAIDs (Diclofenac, Ibuprofen) or Steroids for undefined fever. This causes bleeding.**	**DO Ask every fever customer: *"Do you have stomach pain or vomiting?"* If yes, refuse sale and send to hospital.**	**"I cannot give you a painkiller. It could be Dengue. Take only Paracetamol and go to the clinic."**
Ward Councillors & City Corp.	**STOP relying solely on street fogging (smoke). It does not kill indoor larvae.**	**DO Deploy "Red Teams" to inspect indoor hotspots: Fridge trays, AC drain pipes, and flower vases in high-rise buildings.**	**"Fogging is not enough. We are entering homes today to check water trays."**
Imams & Mosque Committees	**STOP giving generic advice like "Stay safe."**	**DO dedicate the *Khutbah* to a specific action: "Check the mosque's ablution (Wudu) tank and your own fridge tray."**	**"It is a religious duty to remove stagnant water today. Check your home before Asr prayer."**
School Teachers / Headmasters	**STOP ignoring students who put their heads down on desks (lethargy).**	**DO Implement a "Full Sleeve" uniform policy immediately. Allow tracksuits/trousers instead of skirts/shorts.**	**"Wear full sleeves to school. If you feel weak, tell a teacher immediately."**

Category B: Clinical & Medical Responders
These actors determine survival.

Role / Institution	The Critical "STOP" (Do Not Do This)	The Critical "DO" (Mandatory Action)	The Script (What to say)
Village Doctors (Palli Chikitshak)	**STOP** prescribing Antibiotics for viral fever. It wastes money and time.	**DO** Refer any patient with **"Warning Signs"** (Vomiting, Tummy Pain) to the Upazila Complex immediately.	"This fever needs a blood pressure check, not antibiotics. Go to the government hospital now."
Triage Nurses (ER)	**STOP** using "Platelet Count" as the admission criteria.	**DO** Admit based on **Shock Index**: Low Pulse Pressure (<20 mmHg), Cold extremities, or Urine output <500ml/day.	"We are admitting you because your blood pressure is unstable, not because of your platelets."
Diagnostic Centers (Private)	**STOP** price gouging on NS1/CBC tests.	**DO** Display the **Govt. Fixed Price** (e.g., 50/300 BDT) is clearly at the reception counter.	"The government rate is [X] Taka. We are charging that amount."

Message Development and Framing

Core Message Framework:

Color Code	Status	Key Message (The "Hook")	Action Required
🟢 **GREEN**	**Safe**	*"Fever is high? Don't panic. Drink saline."*	Stay home. Drink 3L of fluid daily. Take Paracetamol only.
🟡 **YELLOW**	**Caution**	*"Fever gone but feeling weak? This is the danger zone."*	**Visit Doctor.** If the patient stops eating or is super tired, this is the Critical Phase (Day 4-6).
🔴 **RED**	**Danger**	*"Stomach pain + Vomiting? Ignore the Platelets, Run to Hospital."*	**Immediate Admission.** These are signs of Shock (BP drop), which kills faster than bleeding.

2. Framing for Specific Audiences
For Mothers (The Caregivers)

- **Objective:** Shift focus from "Temperature" to "Urine Output."
- **The Message:** *"Ma, don't worry if the fever is 104°F. Worry if your child hasn't peed in 6 hours. Peeing means they are safe."*
- **Why:** Mothers often stop giving fluids when the fever drops, which is exactly when shock begins.
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For Rural Men (The Decision Makers)

- **Objective:** Prevent dangerous travel to Dhaka.
- **The Message:** *"Your local Upazila Hospital has the exact same saline as Dhaka Medical. Traveling 6 hours to Dhaka with low blood pressure will kill the patient on the road. Stabilize here first."*

For Youth/Students

- **Objective:** Peer monitoring.
- **The Message:** *"If your friend is absent and says 'I'm just weak', go check on them. Weakness after fever is the silent killer."*

Language & Literacy Considerations:

Channel	Format	Content (Draft)

SMS	Text (Bangla)	"জ্বর হলে অবহেলা নয়। ডেঙ্গু পরীক্ষায় নিকটস্থ নগর স্বাস্থ্য কেন্দ্রে যান। প্যারাসিটামল ছাড়া অন্য ব্যথানাশক ওষুধ খাবেন না। - ঢাকা উত্তর সিটি কর্পোরেশন"
Radio	Jingle (15 sec)	(Sound of thunder/rain) "বৃষ্টির পানি জমতে দেবেন না। ৩ দিনে ১ বার, জমা পানি পরিষ্কার। আপনার পরিবারকে ডেঙ্গু মুক্ত রাখুন।"
Poster	Visual	Image of a clean roof vs. dirty roof. Headline: "আপনার ছাদ কি মশার কারখানা? আজই পরিষ্কার করুন।" (Is your roof a mosquito factory? Clean it today.)

4. Rumour & Misinformation Management

Strategy: The "Truth Sandwich" Never repeat the myth alone. Sandwich it between the truth.

Myth 1: "We need Platelets to survive."

- **The Truth:** "Saline saves lives, not platelets."
- **The Myth:** "Many people think low platelets cause death."
- **The Correction:** "Actually, death is caused by 'dry veins' (shock). Platelets will naturally come back up if the blood pressure is strong."

Myth 2: "Papaya Leaf Juice cures Dengue."

- **The Truth:** "You need fluids to keep the heart running."
- **The Myth:** "Papaya juice is popular."
- **The Correction:** "It is okay to drink it, but **NEVER** stop the ORS (Saline). Papaya juice cannot fix low blood pressure."

Myth 3: "Dengue is a Summer disease."

- **The Truth:** "Mosquitoes have adapted to the winter cold inside our homes."
- **The Myth:** "Fever in December is just a cold."
- **The Correction:** "Any fever now is Dengue until tested. Test immediately."

Sample Messages

Source 1: Directorate General of Health Services (DGHS)

Message: "Warning Signs: Severe abdominal pain, persistent vomiting, bleeding from gums, and lethargy. If any of these appear, go to the hospital immediately.".

- **Link:** DGHS National Dengue Guideline (2024-2030) *(Note: Refers to the "Warning Signs" section in the official PDF)*.

Source 2: UNICEF Bangladesh

Message: "Dengue can be fatal, but early treatment saves lives. Do not wait for the patient to bleed. Watch for dehydration signs like dry mouth and no tears when crying."

- **Link:** UNICEF Bangladesh: How to Keep Children Safe from Dengue

Section 5: Channels and Coordination Mechanisms

A multi-channel communication approach is essential to ensure wide reach and message reinforcement. Television and radio remain key channels for mass dissemination, particularly for populations with limited internet access. SMS alerts provide rapid, direct communication of urgent information, while social media platforms enable real-time updates and interactive engagement, especially among younger audiences (WHO, 2017).

Community volunteers and frontline health workers play a crucial role in interpersonal communication, door-to-door outreach, and community engagement. Their involvement increases trust and allows messages to be adapted to local concerns. Effective coordination among government agencies, NGOs, media organizations, and community leaders ensures message consistency and credibility. UNICEF highlights that coordinated communication prevents confusion and strengthens overall outbreak response (UNICEF, 2022).

Section 6: Monitoring, Feedback, and Adaptation

Monitoring and evaluation allow communication strategies to remain responsive and effective throughout the outbreak. Key indicators include message reach, public understanding of dengue prevention and symptoms, and reported behavior changes. Data can be collected through surveys, media analytics, and rapid community assessments (WHO, 2017).

Community feedback mechanisms such as hotlines, social media engagement, and volunteer reports enable two-way communication and help identify gaps, concerns, and emerging rumors. Media and rumor monitoring are critical for detecting misinformation early. Based on monitoring findings, communication actions are adapted by revising messages, changing delivery channels, or intensifying community engagement to address evolving needs (WHO, 2023).

Section 7: Ethics, Inclusion, and Accountability

Ethical emergency communication prioritizes the protection of vulnerable populations while avoiding fear, stigma, and blame. Dengue communication must not single out specific communities or groups, as this can lead to discrimination and reduced cooperation. Instead, messages emphasize solidarity, shared responsibility, and achievable actions (WHO, 2017).

Inclusion and accessibility are ensured by providing information in local languages, using simple wording, visual aids, and formats accessible to people with disabilities. Gender-sensitive communication recognizes different household and caregiving roles and ensures equal access to information. Accountability to Affected Populations (AAP) requires transparency, responsiveness, and meaningful community participation in communication processes (UNICEF, 2022).

Section 8: Supplement – Operational Communication Tools

Emergency communication messages evolve across outbreak phases. During the preparedness and early alert phase, communication focuses on awareness and prevention. In the response and escalation phase, messages emphasize symptom recognition and care-seeking. At peak transmission, life-saving guidance and rumor control are prioritized. During recovery, communication reinforces sustained prevention and community responsibility (WHO, 2017).

Sample SMS: “High fever and body pain may be dengue. Do not delay—visit the nearest health center. Remove standing water weekly to prevent mosquito breeding.”

Sample radio message: “Dengue spreads through mosquito bites, not from person to person. Remove standing water from your home every week. If the fever lasts more than two days, seek medical care immediately. Together, we can protect our community.”

During the first 72 hours of an outbreak, communication begins with official confirmation and preventive guidance within the first 24 hours. Between 24 and 48 hours, regular updates, myth-busting, and intensified media engagement are prioritized. From 48 to 72 hours, communication focuses on reassurance, sustained prevention, and adaptation based on feedback and rumor monitoring (UNICEF & WHO, 2020).

Conclusion

Effective emergency communication is a cornerstone of dengue outbreak response, complementing medical and vector-control interventions. This report demonstrates that clear message development, culturally appropriate framing, coordinated multi-channel dissemination, continuous monitoring, and strong ethical foundations are essential for protecting public health. By aligning with the WHO and UNICEF RCCE frameworks and prioritizing accountability to affected populations, dengue emergency communication can build trust, reduce misinformation, and ultimately save lives.

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